

THE HONEST MOM'S GUIDE

#1
BESTSELLER

A woman with long dark hair is sitting in a wooden rocking chair, holding a newborn baby wrapped in a white blanket. She is looking down at the baby with a gentle expression. The room is dimly lit, with a lamp on the right and a window on the left. The floor is made of dark wood.

Sleep, Baby. *Please.*

A Mom of 6 Shares the Exact Steps That Got Every
One of Her Babies Sleeping Through the Night

SIX & THRIVING
Mother of Six — Still Here at 3 a.m.

Sleep, Baby. Please.

What Actually Worked for All 6 of Mine — Including Twins

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DISCLAIMER: This book is for informational and educational purposes only. The author is not a licensed medical professional, paediatrician, or certified sleep consultant. Strategies are based on personal experience raising six children and extensive research. Every baby is different, and results will vary. Always consult your paediatrician before making changes to your baby's sleep routine, especially if your baby has any medical conditions.

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SIX & THRIVING

Practical parenting resources for the exhausted and the hopeful.

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INTRODUCTION

To the Parent Reading This at 3 a.m.

Let me guess. It's somewhere between midnight and 4 a.m. You're sitting in the dark — probably in a rocking chair, maybe on the nursery floor — holding a baby who should, by every logical measure, be completely exhausted. And yet here you both are.

You've tried everything. The shushing. The swaddling. Patted, bounced, sung your way through songs you didn't know you remembered. And still — still — this tiny human will not sleep.

I've been there. Six times.

My name is behind the pen name Six & Thriving — and yes, I have six kids. A set of twins tucked into the middle, a strong-willed firstborn, a sensitive soul, an adventurous spirit, and the baby who nearly broke me before finally sleeping through at 14 weeks.

Every single one came as a unique sleep puzzle. What worked for baby #1 was useless for baby #3. The method that felt wrong with my twins turned out to be exactly right. I read every book, every article, every 2 a.m. forum post. Then I built this system.

**WHO THIS BOOK IS FOR**

Your baby (newborn–24 months) is not sleeping and you have no idea why. You've tried 'just leaving them to cry' and it felt wrong. You're so tired you've Googled 'is it safe to sleep standing up.' You want a real plan from a real parent — not clinical theory.

MY PROMISE TO YOU

By the end of this book, you'll have a step-by-step plan you can start tonight. Sleep is coming — for both of you. I promise.

FAST-TRACK

Tonight's Plan

If you read nothing else, read this page first.

These six steps are the foundation of everything in this book. Do these tonight — read the rest when you have twenty quiet minutes.

1

Check the room right now

Pitch black? White noise running? Temperature 68–72°F / 20–22°C? Fix it before anything else. This single change can shift the entire night.

2

Find your baby's awake window

Turn to the Awake Windows table in Ch. 3. Note the maximum time your baby should be up. Set a timer. Don't let them go past it.

3

Write tonight's bedtime routine — in order

Bath → lotion → pyjamas → feed (not to sleep) → song → crib awake. Same sequence every night. Stick it on the door.

4

Decide your response plan before night falls

Choose your method from Ch. 6 right NOW. Your 2 a.m. brain won't make good decisions. Write it down.

5

Pause before you go in

When you hear a sound, wait 2–3 minutes. Many babies cycle through light sleep and resettle. Rushing in fully wakes them.

6

Put them into the crib AWAKE

Drowsy is fine. Already asleep is not. This single skill changes everything. If they always fall asleep in your arms, Ch. 4 is your priority.

DONE IS BETTER THAN PERFECT

Pick one thing from this list and do it tonight. Progress starts with a single consistent step — not flawless execution of all six at once.

01

Why Your Baby Won't Sleep — And It's Not Your Fault

*Once you understand the WHY, the WHAT TO DO
becomes so much clearer.*

CHAPTER ONE

Babies Aren't Designed to Sleep Like Adults

Here's the truth nobody leads with: babies are biologically wired to wake up frequently. This is not a design flaw. It is a survival mechanism. Frequent waking means frequent feeding (essential in early infancy), and lets your baby alert you if something is wrong.

Adult sleep cycles last roughly 90 minutes. We move through light sleep, deep sleep, and REM. When we hit the light stage, we barely notice — we roll over and drift back. A baby's sleep cycle is much shorter — around 45–50 minutes — and when they hit the light stage, many of them fully wake up and need help getting back down.

The 5 Most Common Reasons Babies Won't Sleep

1. Sleep Associations — The Sneaky Culprit

Whatever your baby associates with falling asleep, they will need at every wake-up. Always nurses to sleep? They'll need to nurse at 1am, 3am, 5am. This is the #1 reason babies who 'used to sleep great' suddenly start waking constantly — usually around 4 months.

2. Overtiredness — The Counter-Intuitive Trap

You'd think a more tired baby sleeps better. They don't. Overtiredness triggers a cortisol response — the stress hormone — that makes it HARDER to fall and stay asleep. An overtired baby is a wired baby.

3. Under-tiredness — Yes, This Is Real

If bedtime is too early or naps are too long, your baby simply isn't tired enough. I made this mistake with baby #4 — eager to get bedtime moving earlier and ended up with a baby ready to party at 7pm.

4. Environmental Factors

Too much light, too much noise, room too warm or cold — the sleep environment matters more than most parents realize. The room your baby sleeps in is doing quiet work, either FOR you or AGAINST you.

5. Developmental Leaps & Growth Spurts

Just when you crack the code, baby hits a leap and everything goes sideways. This is normal. It is temporary. You haven't broken anything — they're growing. Common leap points: 4 months, 8–10 months, 12 months, 18 months, 24 months.

REMEMBER THIS

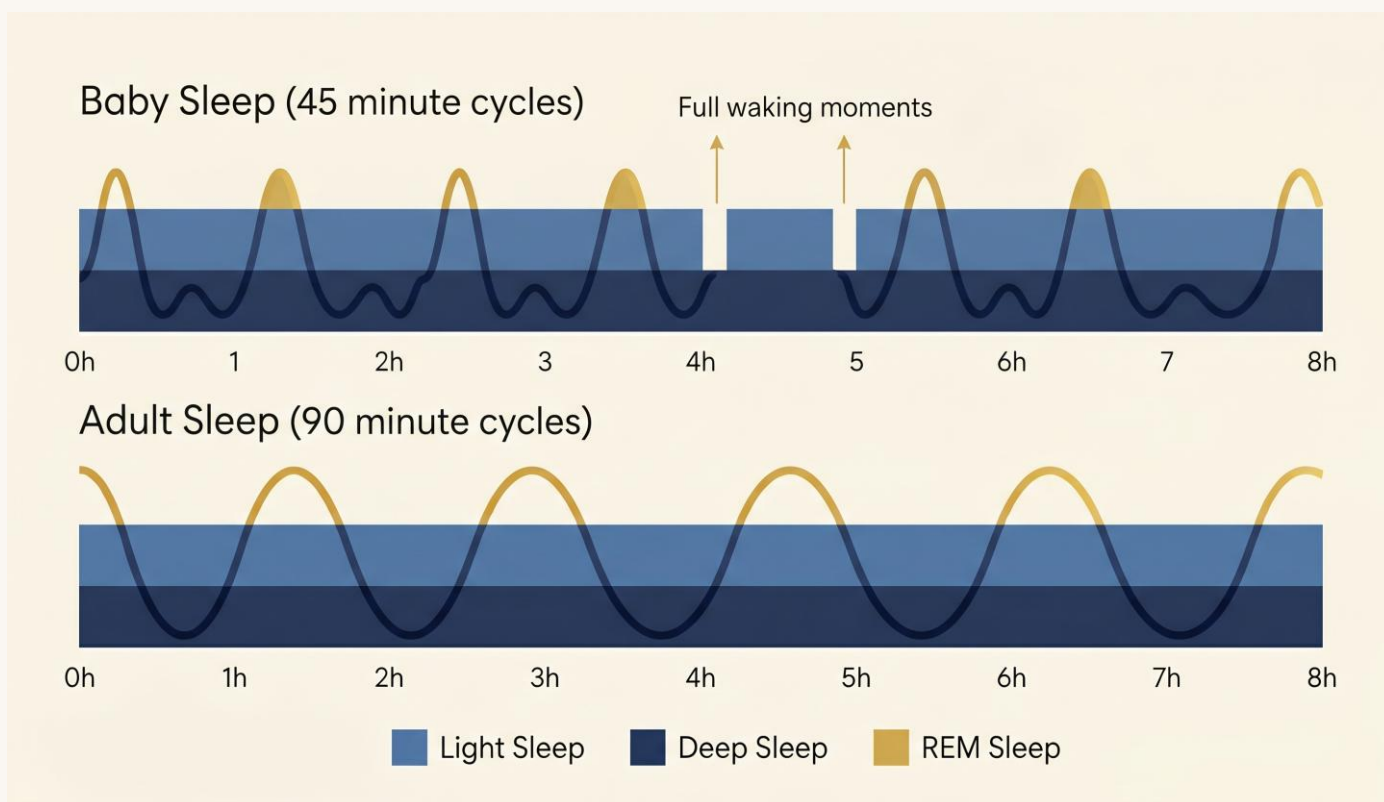
A baby waking at night is not failing. A baby waking at night is doing exactly what their biology expects of them. Your job — and this book's job — is to gently teach them a new way.

CHAPTER ONE

And Now — The Part Where I Tell You It's Not Your Fault

Hear this before we go further: you did not cause this by holding your baby too much, by responding to their cries, or by nursing them to sleep. You responded to your baby's needs. That is parenting. The habits that formed are just habits — and habits can be changed.

I've met parents who felt crushing guilt over sleep struggles. Parents convinced they had 'done it wrong.' You haven't. You're here, reading this, looking for answers. That IS doing it right.



45

MIN

Baby's sleep cycle

4-6

MONTHS

Earliest training age

7

NIGHTS

Most see real change

TRIED THIS BEFORE AND IT DIDN'T WORK?

You're not starting from zero — you're starting from experience. Most common reasons it failed:

- Wrong method for baby's temperament
- Inconsistency (especially Night 3)
- Environment wasn't optimised
- Started during a regression
- Partner not aligned

This time, you have the full picture. Keep reading.

02

The Sleep Science Every Parent Actually Needs to Know

*I'm not going to bury you in neuroscience.
These are the pieces I wish someone had handed me on day one.*

CHAPTER TWO

Sleep Pressure & The Sweet Spot

Sleep pressure is the building pressure to sleep that accumulates the longer your baby is awake. It's driven by adenosine, a chemical that builds up in the brain. The longer awake, the more adenosine, the more pressure to sleep.

If you put baby down BEFORE enough sleep pressure has built up — they won't settle. If you wait TOO LONG — cortisol kicks in and you've got an overtired wired baby.

The sweet spot — where sleep pressure is high enough to make settling easier but cortisol hasn't taken over — is your new best friend.

Cortisol vs. Melatonin: The Two Sides of the Battle

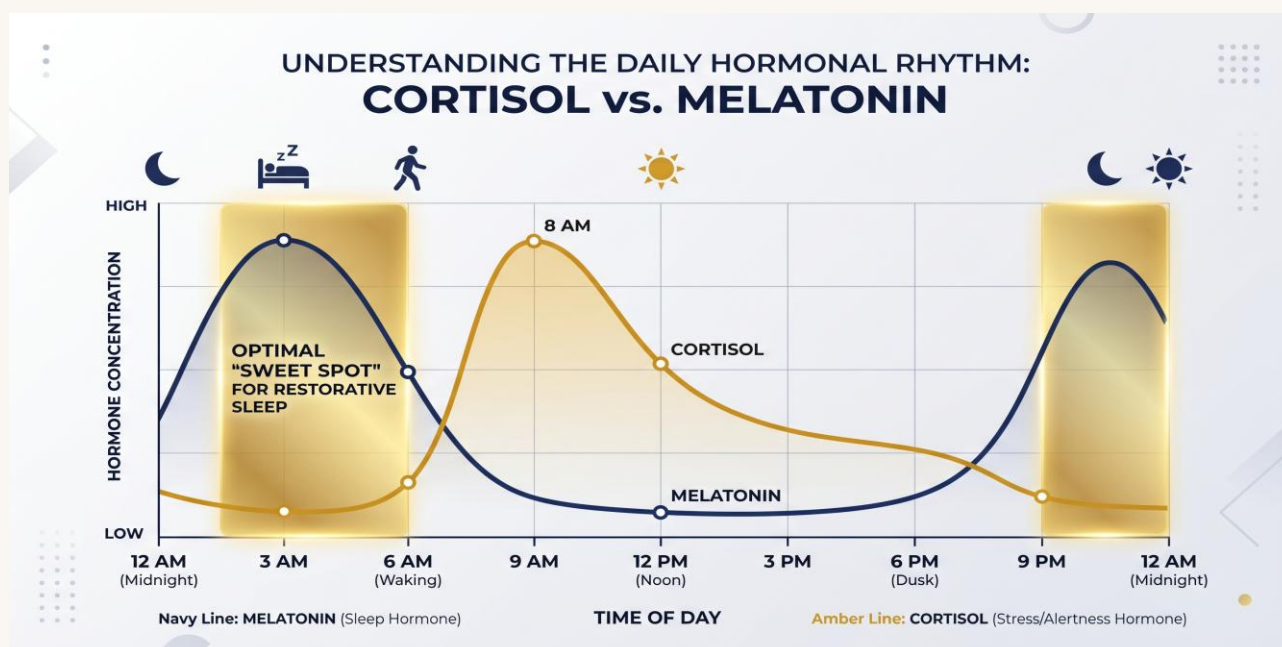
Melatonin says: SLEEP. Cortisol says: STAY AWAKE. When baby is past their optimal awake window, cortisol rises. That's why an overtired baby seems wired — they literally are. Their body is pumping out a stress hormone to keep them going.

The Circadian Rhythm — Your Baby Has One Too

Newborns don't arrive with a set circadian rhythm — it takes 6–8 weeks to develop. By then, you can use light strategically:

- Bright natural light in the morning sets the circadian clock
- Dimming lights in the hour before bed signals sleep is coming
- Darkness during the night keeps the clock from resetting too early

Melatonin is suppressed by light and released in darkness. A nightlight that seems dim to you may be working against your baby's melatonin release.



TIMING TIP

Watch for sleepy cues 10–15 minutes BEFORE the end of the expected awake window and start the wind-down then. Don't wait for yawning and eye-rubbing — by then, cortisol may already be rising.

CHAPTER TWO

The 45-Minute Intruder

Understanding baby sleep cycles — the chart that changed everything for me.

STAGE	WHAT'S HAPPENING	DURATION
NREM 1–2 (Light)	Drowsy, easily woken, may startle or twitch	10–15 min
NREM 3 (Deep)	Harder to wake, breathing slows, full body relaxation	15–20 min
REM (Active)	Eyes flutter, facial movements, most dreams occur here	10–15 min
Transition	Baby surfaces, may fully wake without sleep associations	~45 min total

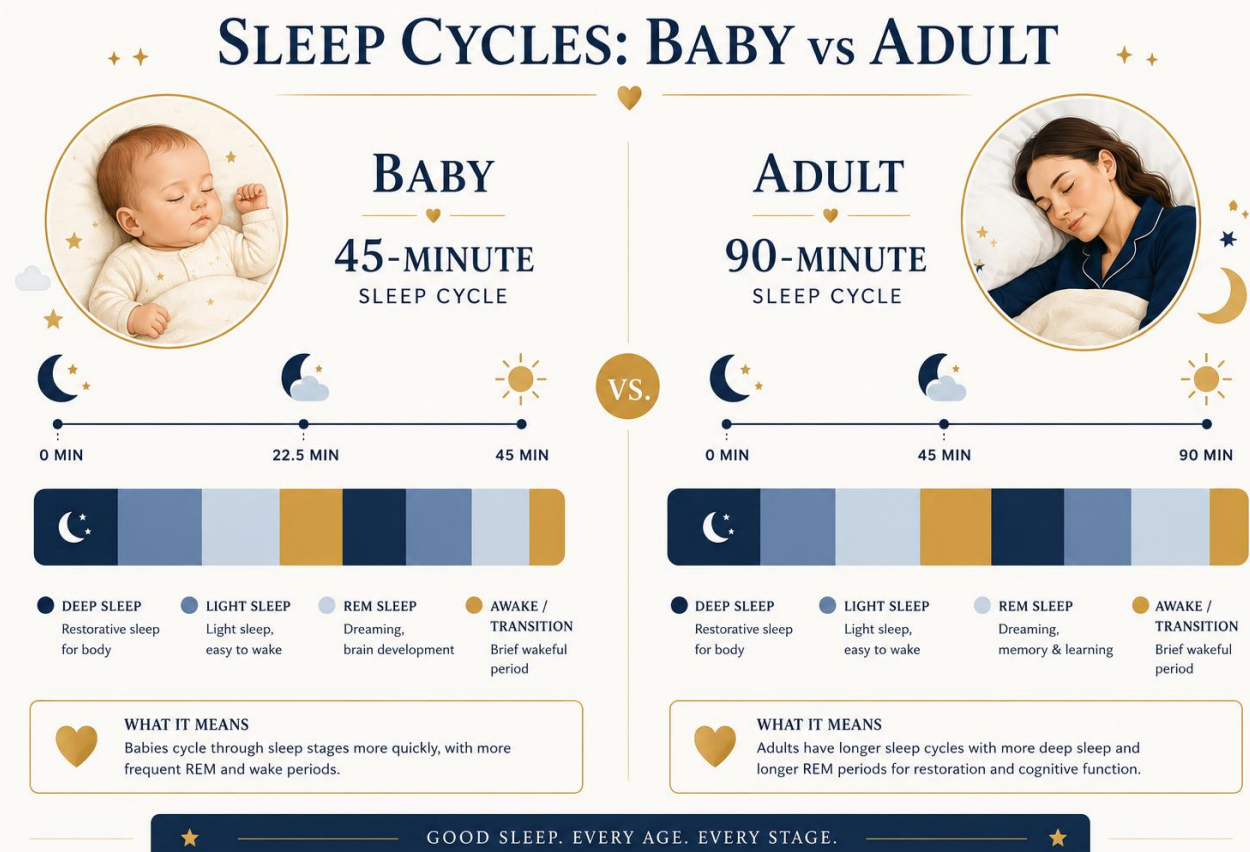
When baby surfaces from a cycle and doesn't know how to resettle, you get the '45-minute nap' — they wake, can't link cycles, end of nap. Teaching a baby to resettle (the core of sleep training) is teaching them to get back through the transition on their own.

What 'Sleeping Through the Night' Actually Means

Reset expectations: 'sleeping through' is clinically defined as a 5–6 hour stretch. NOT 7pm to 7am. A 3-month-old doing a 6-hour stretch IS sleeping through the night. That is a win.

By 6 months: 8–10 hours is achievable. By 9–12 months: 10–12 hours becomes normal. It's a process, not a switch.

Progress over perfection. Every longer stretch is a victory.



03

Reading Your Baby's Sleep Cues Before It's Too Late

If sleep timing were a game, most of us are playing it on hardest difficulty because we don't know the rules. Reading cues is learning the rules.

CHAPTER THREE

The Sleep Cue Traffic Light

Stage 1 Green Light



- slowing movements
- quiet gaze
- single yawn

**Start your
routine NOW**

Stage 2 Yellow Light



- yawning frequently
- eye rubbing
- ear pulling
- fussier

**Begin routine
immediately**

Stage 3 Red Light



- crying
- arching back
- wired and hyper

**Damage
control mode**

RED-LIGHT RECOVERY

When you hit a red light: dim the lights, lower your voice, add extra steps to the wind-down. It'll take longer but you can still get there. Give yourself and your baby grace. Tomorrow: aim for green.

THE 7-DAY SLEEP LOG

Track wake times, sleep times, mood for 5–7 days. You don't need an app — a notebook works. Just note: wake time, nap start/end, bedtime, night wakings, one-word mood. Patterns emerge within a week. Patterns give you power.

REFERENCE

Awake Windows by Age

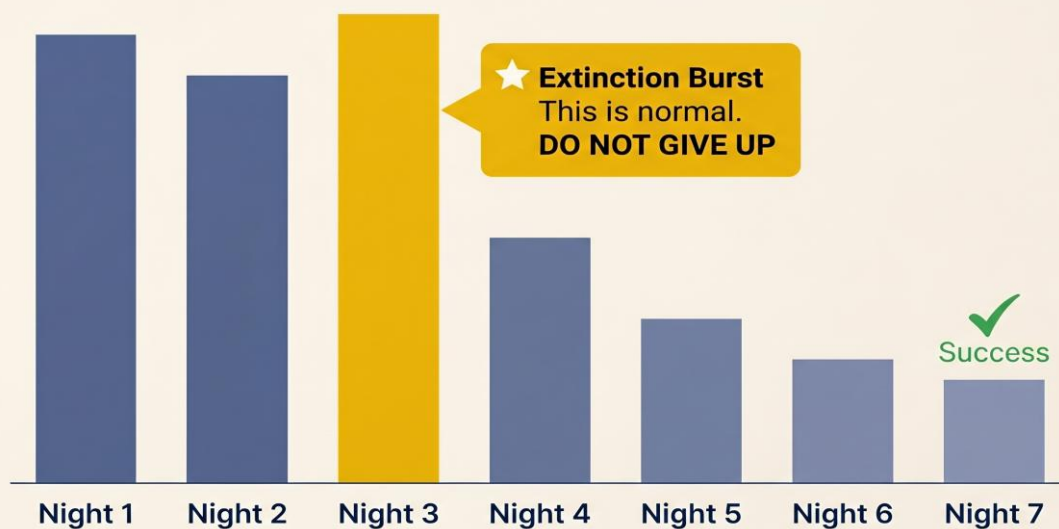
These are averages. Your baby may run 15 minutes either side. The shape is what matters.

AGE	AWAKE WINDOW	NAPS/DAY	NOTES
0–4 weeks	45–60 min	4–5	Watch for early cues; very short windows
1–2 months	60–90 min	4–5	Gradually extending
2–3 months	75–90 min	4	Circadian rhythm starting
3–4 months	90 min – 2 hrs	3–4	4-month regression often hits here
4–6 months	2 – 2.5 hrs	3	More predictable scheduling begins
6–8 months	2.5 – 3 hrs	2–3	Transition to 2 naps often starting
8–10 months	3 – 3.5 hrs	2	2-nap schedule solidifies
10–12 months	3.5 – 4 hrs	2	Approaching toddler territory
12–18 months	4 – 5 hrs	1–2	Transitioning to 1 nap
18–24 months	5 – 6 hrs	1	1 solid nap, longer nights

THE MOST COMMON TIMING MISTAKE

Putting baby down at a fixed clock time regardless of when they woke from their last nap. Count **FORWARD** from wake time, not **BACKWARD** from a target. 'We always do 7pm' will fail you.

Crying Duration in Minutes



TECHNIQUE • CHAPTER THREE

The Crib Hour

The nap technique that teaches sleep-cycle linking.

If your baby consistently wakes after 30–45 minutes of every nap and cannot resettle — they are a one-cycle napper. Their brain is surfacing from the first sleep cycle and does not yet know how to link into the next one. The Crib Hour teaches them.

What it is

After your baby goes down for a nap, give them **ONE FULL HOUR** in the crib — regardless of when they wake. If they surface at 28 minutes, leave them (as long as they're not in distress) until 60 minutes have passed from when they first went down. Do not collect them at the first sound.

Why it works

A baby left in a calm, dark, white-noise environment after waking will often resettle from boredom within 5–10 minutes — linking naturally into a second cycle. Each time, the brain practices independent cycle-linking. Within **ONE WEEK** of consistent practice, many 30-minute nappers become 60–90 minute nappers.



The Rules at a Glance

Calm or intermittent sounds	Leave them. Self-settling is in progress.
Escalating, distressed cry	Go in, offer minimal comfort, resettle and leave.
After 60 minutes	Always go in. End the nap regardless.
Environment required	Same as night: full blackout, white noise running.
Age to begin	From around 4 months, when sleep cycles mature.

THE CRIB HOUR IS NOT CRY-IT-OUT

You are not ignoring your baby. You are giving them the space to practice a skill. Watch and listen throughout. If something feels wrong, always go in.

04

Building a Bedtime Routine That Actually Sticks

A bedtime routine is not just cute events before sleep. It is a neurological signal. Done consistently, the routine itself triggers the brain's sleep response.

CHAPTER FOUR

The Pavlov Effect for Babies

Pavlov's dogs learned to salivate at a bell because the bell reliably predicted food. Your baby can learn to feel sleepy at the sight of a bath or sound of a lullaby — because those things reliably predict sleep.

The key word is CONSISTENT. The routine doesn't need to be elaborate. It doesn't need lavender wash or a \$300 sound machine. It needs to be the SAME, in the SAME ORDER, every single night.

The Four Components of an Effective Bedtime Routine

1. The Transition Signal

Marks the beginning of wind-down. Turning off TV, closing blinds, saying a phrase like 'Okay, it's sleepy time!' Tells the brain: the active part of the evening is over.

2. The Wind-Down Activity

Lower stimulation actively. A warm bath is the gold standard — not because of the lavender, but because immersion in warm water raises body temperature, and the subsequent COOLING after the bath triggers the sleep drive. AVOID: screens, bright lights, rough play, exciting visitors.

3. The Feeding or Comfort Step

Feed them here — but here's the critical piece: do NOT let them fall asleep during the feed. If they always fall asleep at breast or bottle, the feed BECOMES a sleep association. Feed enough to be satisfied, then move to the sleep space. Goal: drowsy but awake.

4. The Sleep Trigger

The last thing before you leave the room. A specific song (sung live, not a device), a brief cuddle in the crib, a phrase like 'I love you, goodnight, sleep well.' The final cue. Keep it short. Keep it warm. Keep it the SAME.

Night One Starts Tomorrow — Set Up Today 🚀

- ☐  **Blackout curtains out**
Room fully blacked out
- ☐  **Temperature set to 68–72°F**
Temperature set to 68–72°F
- ☐  **Routine written and posted on door**
Routine written and posted on door
- ☐  **Partner fully briefed and aligned**
Partner fully briefed and aligned
- ☐  **Sleep log set up and ready**
Sleep log set up and ready

Done is better than perfect. Start tonight.

ROUTINE TIMING

The whole routine should take 20–30 minutes. Much shorter and it's not long enough to wind down. Much longer and you risk baby falling asleep DURING the routine instead of in the crib.

CHAPTER FOUR

Sample Routines by Age

Newborn–3 Months	3–6 Months	6–18 Months
1. Dim lights 30 min before sleep 2. Sponge bath or wipe-down 3. Feed in quiet dim room 4. Swaddle while still calm/awake 5. Hold & rock briefly with shushing 6. Place in crib drowsy but awake 7. Hand on chest briefly, then leave	1. Transition signal: lights dim 2. Warm bath (5–10 min) 3. Lotion massage (5 min) 4. Pyjamas and sleep sack 5. Feed in dim room — keep awake 6. 1–2 short songs 7. Into crib drowsy but awake	1. Clean-up cue ('toys to bed!') 2. Bath 3. Pyjamas and sleep sack 4. 2–3 short books in chair 5. Feed/milk (not to sleep) 6. Goodnight phrase 7. Into crib, white noise on

The Most Important Rule: Same Order. Every Night.

Rotate the songs. Switch the books. But the ORDER stays the same. The order is the signal. Once your baby's brain maps it, each step starts to turn down their alertness. By the last step, they're already halfway to sleep.

Give it 7 days of consistency before evaluating. You're building a neural pathway, and that takes repetition.

The Foundation Bedtime Routine



Start tonight

05

Setting the Stage: The Sleep Environment

*You can have the perfect routine and the perfect timing —
and still have a rough night if the environment
is working against you.*

CHAPTER FIVE

Darkness, Sound & Temperature

Darkness: The Non-Negotiable

TRUE darkness — where you genuinely cannot see your hand in front of your face — is the single most impactful environmental change most families can make. Melatonin is significantly reduced by even low-level light. That little nightlight that seems harmless? If your baby can see it, it's working against their sleep hormone.

- Use blackout curtains or blinds — not 'room darkening' — true blackout
- Cover any glowing LEDs on monitors with electrical tape
- If you need a nightlight for safety during feeds, use a RED-spectrum light
- The hallway light coming under the door matters — use a draft stopper

White Noise: Your Baby's Sleep Soundtrack

The womb is loud. Really loud — about 80–85 decibels, similar to a vacuum cleaner. So the perfectly silent nursery that feels peaceful to YOU actually feels startlingly quiet to a newborn.

White noise helps two ways: recreates a familiar sound environment, AND masks household sounds (dog barking, sibling running, delivery at the door) that wake babies in light sleep.

Temperature: The Goldilocks Factor

Ideal room temperature: 68–72°F (20–22°C). Too warm and babies rouse easily. Too cold and they can't maintain comfortable sleep. Babies can't regulate body temperature like adults — dress them based on the ROOM TEMP, not what feels comfortable to you.

Rule of thumb: dress your baby in ONE more layer than you'd wear comfortably to sleep.



WHITE NOISE SETUP

Play at 60–65 decibels — similar to shower noise — measured where baby's head will be. Don't put the machine in the crib or right next to the ear. A few feet away, running ALL night, is the goal. Fan, static, rain, brown noise — all work. Pick one and stick with it.

CHAPTER FIVE

The Crib & The Room

The Crib Setup: Safe and Sleep-Inducing

AAP guidelines: firm flat mattress, fitted sheet, NOTHING ELSE. No bumpers, pillows, stuffed animals, or positioners. This isn't just about safety — a clear, simple sleep space is also better for sleep. Babies don't need stimulation in their sleep environment.

After 12 months (when SIDS risk drops significantly), you can introduce a comfort object — a small lovey or stuffed animal. Many babies form genuine attachments that help them resettle at night.

The 'Away From the Action' Principle

If possible, baby's sleep space should feel removed from the busyness of the house. This doesn't mean silent — ambient household noise is fine and helps babies habituate. But a nursery right next to the kitchen where dishes are clattering, or adjacent to a loud playroom, is a harder environment.

Small space? White noise becomes more critical. Room-sharing? Use a partition or curtain to create visual separation, and keep light minimal during night feeds.

Room-Sharing Note

The AAP recommends ROOM-sharing — but NOT bed-sharing — for the first 6–12 months. This reduces SIDS risk while keeping baby in a safe, separate sleep surface. When transitioning out of your room, do it during a stable period — not during illness, travel, or a regression.

The Crib Hour



If distressed: go in. If calm: wait.

THE BOTTOM LINE

The room doesn't need to be perfect. It needs to be CONSISTENTLY set up for sleep. Done is better than perfect here. Make the changes you can make tonight. Plan the rest for this weekend.

06

The Big Sleep Training Debate: Finding What Fits YOUR Baby

*I've used almost every method across my six kids.
Some worked beautifully. Some didn't fit my baby's temperament.
Here's the full picture.*

CHAPTER SIX

Method 1: Extinction (Cry It Out)

What it is: You put baby down awake and don't return until morning (or a pre-set feeding time). No checks, no response unless there's genuine concern.

The research: Multiple studies show extinction does NOT cause long-term harm to emotional development, cortisol levels, or attachment when done after appropriate age (4–6+ months). Tends to produce results fastest — often 3–5 days.

Who it fits: Families struggling severely and needing fast results. Babies who are highly persistent — who actually escalate MORE with parental check-ins. Parents who can stay consistent without exception.

The honest truth: It is hard. Even knowing the research, sitting in another room while your baby cries is one of the hardest things you'll do. Make sure both partners are aligned BEFORE starting. Have a plan for the first few nights.

Method 2: Ferber (Graduated Extinction)

What it is: Check on baby at gradually increasing intervals — 3 min, 5 min, 10 min, 15 min (cap there) — without picking up. Verbal reassurance only.

Day-by-Day Ferber Schedule:

DAY	1ST WAIT	2ND WAIT	3RD WAIT	MAX WAIT
Day 1	3 min	5 min	10 min	10 min
Day 2	5 min	10 min	12 min	12 min
Day 3	10 min	12 min	15 min	15 min
Day 4+	12 min	15 min	17 min	17 min

WHEN FERBER ISN'T A FIT

If your baby **ESCALATES** with each check-in instead of calming between them — Ferber may not be right. They see you, you leave again, they're more upset. Switch to extinction or Chair Method.

DURING CHECK-INS

Stay 30 seconds max. Pat their back briefly. Say a calm phrase: 'I love you. It's sleep time.' Do NOT pick up. Do NOT turn on lights. Do NOT engage. Then leave.

CHAPTER SIX

The Gentler Methods

Method 3: Chair Method (Sleep Lady Shuffle)

What it is: Sit next to the crib until baby falls asleep. Every few nights, move your chair progressively further from the crib until you're out of the room.

Week 1: Chair beside crib. You're there but not engaging. Brief shushing/reassurance only.

Week 2: Chair halfway across the room.

Week 3: Chair at the doorway.

Week 4: Chair just outside, door open.

Week 5: Out of sight, door open.

Week 6: Door closed.

Best for: Sensitive babies who need parental presence. Parents not ready to step back fully. Slower (2–4 weeks) but gentler.

Method 4: Fading

What it is: Gradually reduce your involvement over time. Less rocking each night, less nursing, less of the current sleep association — until baby is independent.

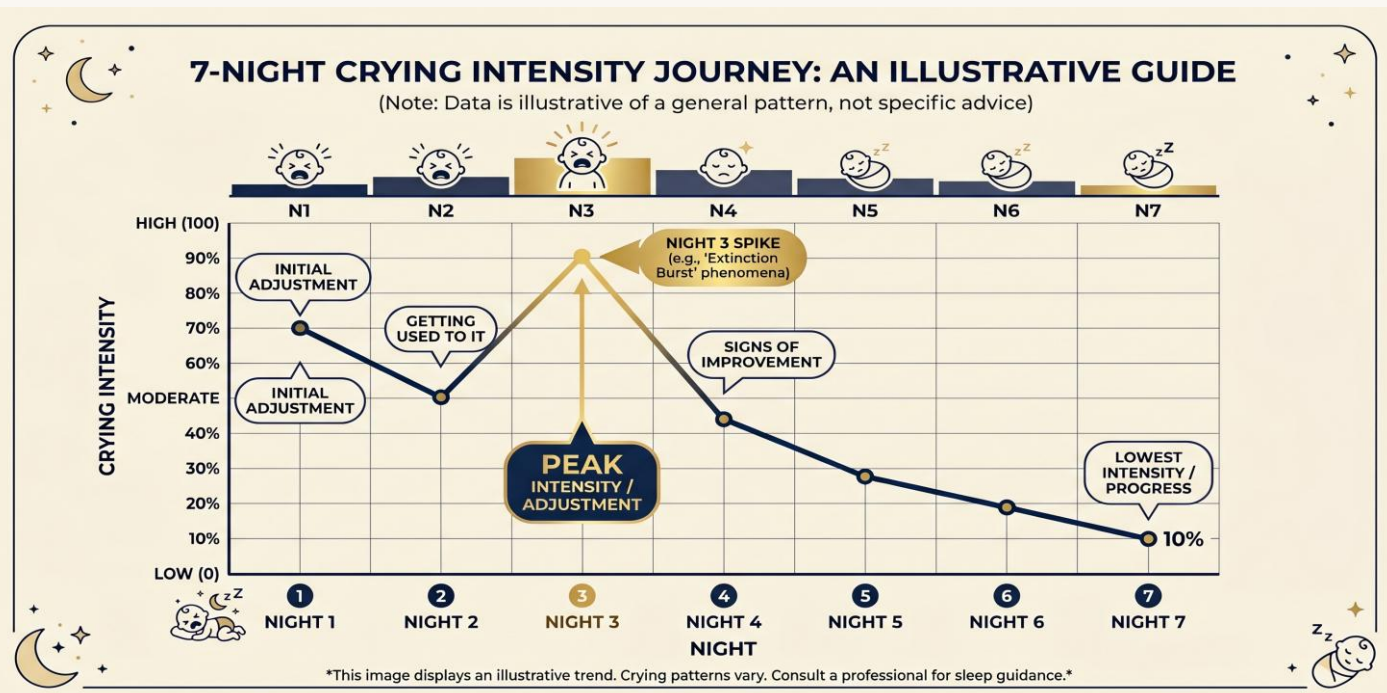
Example: If you currently rock for 20 min — Night 1: 18 min. Night 2: 16 min. Night 3: 14 min. Continue reducing by 2 min each night until you're rocking for 0.

Best for: Gentlest method. Adaptable babies tolerant of gradual change. Parents who want low/no crying. Easier to stall out — full removal of association required.

Method 5: Pick Up / Put Down

What it is: Put baby down. If they cry, pick up to calm. Put down again. Repeat until they fall asleep.

Best for: Younger babies (3–5 months) needing high support. Parents who can't tolerate extended crying. Can take a long time per night with older or more persistent babies.



THE TRUTH ABOUT METHODS

There is NO universally 'best' method. The best one is the one you can commit to consistently. A gentle method done inconsistently is LESS effective — and harder on baby — than a stricter method done with unwavering consistency.

CHAPTER SIX

Choose Your Method

Direct comparison — pick what fits your family.

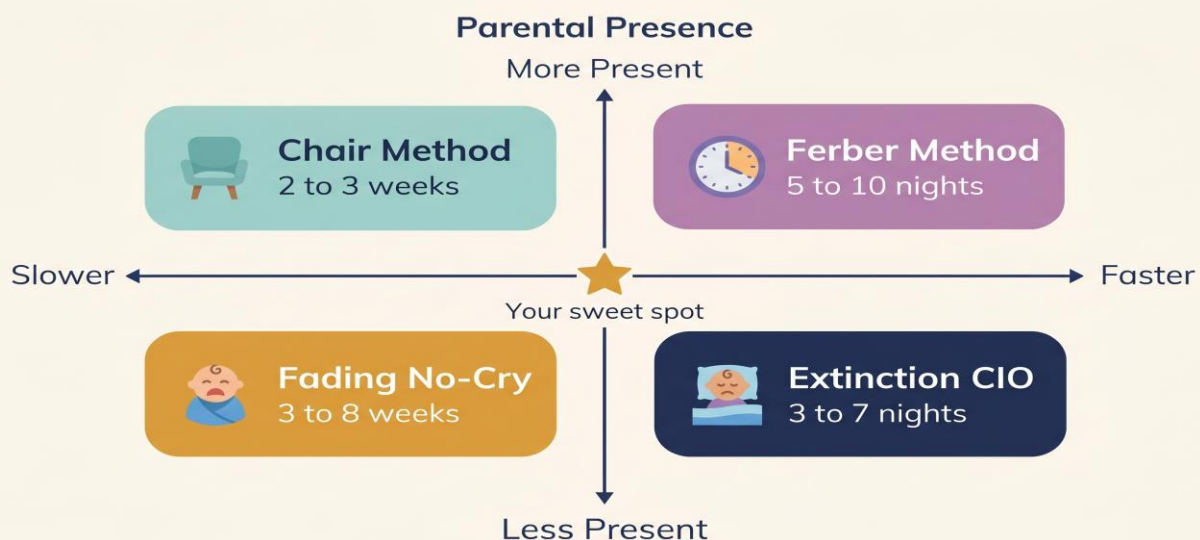
METHOD	CRY LEVEL	SPEED	BEST FOR
Extinction (CIO)	High	3–5 days	Persistent babies; severe sleep deprivation
Ferber	Moderate	5–7 days	Most temperaments; parents wanting to do something
Chair Method	Low–Mod	2–4 weeks	Sensitive babies; gradual approach
Fading	Low	3–4+ weeks	Gentlest; adaptable babies; no-cry priority
Pick Up/Put Down	Low	Variable	3–5 month olds; high-support preference

When to Start Sleep Training

Most paediatricians and consultants agree 4–6 months is the earliest most babies are developmentally ready. Before that, night feeding is often genuinely necessary and the nervous system isn't mature enough for independent settling.

Do NOT start during: illness, teething peaks, a move, travel, or a developmental leap. Wait for a stable 2-week window with no major disruptions ahead.

Pick your method. Pick your start date. Commit. Consistency is everything.



DECISION SHORTCUT

If you need fast results AND can stay consistent → Extinction. If you need 'something to do' during crying → Ferber. If your baby is very sensitive → Chair Method. If you want zero crying → Fading. Pick one TONIGHT. Don't second-guess.

07

Night Wakings & The Reset Protocol

How you handle wakings makes the difference between waking that's gradually shrinking and waking that stays stubbornly in place for months.

CHAPTER SEVEN

The Wait-Before-You-Go Principle

This is hard when every instinct says GO. But here's what I've learned: many babies who wake and make noise at night are NOT fully awake. They're cycling through a light sleep phase. If you go in immediately, you fully wake them. If you wait 2–3 minutes, there's a real chance they'll settle on their own.

This doesn't mean letting your baby scream while you count seconds. It means PAUSING before acting — listening for whether sounds are escalating or quieting. Within days, you'll know the difference between 'I'm cycling' noises and 'I genuinely need you' cry.

The Lovey Strategy

A lovey is a small soft comfort object — stuffed animal or small blanket with soft toy. When introduced consistently, many babies transfer their comfort-seeking from a parent (or breast/bottle) to the lovey.

How to build the association: Keep the lovey near your body or the nursing area during daytime feeds so it picks up your scent. Introduce at bedtime and naps. Once associated, baby can find and hold the lovey at 3am without needing you. APPROPRIATE FROM AROUND 12 MONTHS.

Night Feeding: Nutritional vs. Habitual

AGE	TYPICAL NIGHT FEEDS	GUIDANCE
0–3 months	2–4 per night	Not yet — these are nutritional
3–4 months	1–3 per night	Work on timing & drowsy-but-awake first
4–6 months	1–2 per night	Begin gradual reduction if weight is good
6–9 months	0–1 per night	Most babies can handle 0 with paed OK

Night Feeds by Age: When to Reduce

0–3 months  Not Yet — These Are Nutritional

3–4 months  Start Reducing Gradually

4–6 months  Start Reducing Gradually

6–9 months  Most Babies Can Handle Zero

9+ months  Most Babies Can Handle Zero



CHAPTER SEVEN

Reducing Night Feeds & Avoiding Sleep Debt Traps

How to Reduce a Night Feed Without Going Cold Turkey

BOTTLE FEEDING: Reduce the amount offered at the target waking by 1 oz every 2–3 nights. Your baby gradually stops waking for a feed that isn't worth the effort.

BREASTFEEDING: Gradually reduce feeding TIME at the target waking — by about 1 minute every 2–3 nights. So if currently 8 min, go to 7, then 6, etc. As the feed becomes less substantial, the waking often disappears.

Always check with your paediatrician before reducing night feeds, especially under 6 months or with weight concerns.

Habits to Avoid After the First Few Months

I call these 'sleep debt traps' — patterns that feel like solutions in the moment but create harder problems over time:

- Feeding to sleep after 4 months — every waking will require a feed to resettle
- Rocking fully to sleep — they'll need rocking back at every cycle
- Bringing baby into your bed as a rescue — this becomes expected very quickly
- Motion sleep for all naps — car, pram, swing sleep doesn't build same sleep pressure

None of these make you a bad parent. They make you a HUMAN parent who did what was necessary to survive. Now let's gently undo them.



PROTOCOL · CHAPTER SEVEN

The Reset Protocol

When everything has slipped — start here. The 5-step rescue plan.

1

Diagnose the root cause

Is it sleep association? Timing? Environment? Partner inconsistency? Regression? Pick ONE primary culprit before fixing anything.

2

Fix the environment first

Blackout, white noise, temperature 68–72°F. These are FREE and IMMEDIATE wins. Do this tonight before anything else.

3

Reset the routine to original order

Strip back any steps that have crept in. Bath → lotion → pyjamas → feed → song → crib. Print it. Stick it on the door.

4

Commit to 5 consecutive nights

Same method, same response, no exceptions. Five consecutive consistent nights resolves most slides completely. No deviations.

5

Track and compare

Use the 7-Day Tracker. Compare Night 5 to Night 1. Time to settle, number of wakings, total sleep. The data motivates you when memory fails.

ONE SLIP DOES NOT ERASE PROGRESS

A single difficult night is NOT a regression. Three or more consecutive difficult nights with a CHANGING response pattern is. Know the difference. Then act.

REGRESSION REFERENCE

• 4 months: circadian shift • 8–10 months: separation anxiety • 12 months: walking • 18 months: autonomy • 24 months: imagination. Hold the line during regressions — they pass within 1–4 weeks.

08

The Six Sleep Personalities (And How I Cracked Each One)

*Six babies. Six completely different little humans.
See if your baby recognizes themselves
in one of these.*

CHAPTER EIGHT

Personalities 1–3

1. The Snacker

Who they are: Feeds little and often. Has learned to associate feeding with sleep. Wakes frequently throughout the night — not from hunger, but because the breast or bottle is their sleep cue.

What works: Gradually consolidate feeds (fewer but larger). Use the 'feed, then wake slightly, then settle' sequence at bedtime to break feed-to-sleep association. Introduce a pacifier or lovey as a substitute comfort.

2. The Overthinker

Who they are: Alert, curious, deeply interested in everything happening around them. They don't want to miss anything. Fights sleep even when clearly exhausted because the world is too interesting.

What works: An EARLIER bedtime than you'd think necessary. Extremely low-stimulation wind-down (no mobiles, toys, interactive anything during the last 30 minutes). Often a darker, quieter room than average.

3. The Sensitive Soul

Who they are: Picks up on everything — change in your tone, a slightly different bedtime, a new smell in the room. Easily startled, emotionally attuned, prone to more frequent waking during periods of change or stress.

What works: Extraordinary routine consistency. A calm, regulated caregiver (hard when you're exhausted). A gentler sleep training approach (Fading or Chair over Extinction). Especially mindful of developmental leaps.

THE PERFECT NURSERY AT NIGHT



CHAPTER EIGHT

Personalities 4–6

4. The Night Owl

Who they are: Genuinely seems to be running on a later circadian schedule. Not tired at 7pm — tired at 9 or 10pm — and they'll let you know if you try to put them down before they're ready.

What works: Gradually shift the schedule earlier over 2–3 weeks (15 min every 2–3 days). Maximize MORNING light exposure immediately upon waking. Shorten the LAST nap of the day to build more sleep pressure by evening.

5. The Catnapper

Who they are: Never naps for more than 30–45 minutes. Surfaces from the first sleep cycle and wakes fully — leaving you with a baby who's not fully rested and a window of time too short for anything.

What works: USE THE CRIB HOUR (Chapter 3). Be in the room at the 40-minute mark to attempt resettling before they fully wake. Optimize the sleep environment — darkness and white noise matter enormously for naps. For some Catnappers, accept a 3-nap schedule that accounts for shorter individual naps.

6. The Party Animal

Who they are: Wakes between 1–4am ready to go. Not crying — happy. Chatting. Playing with their feet. They're having a great time. You are not.

What works: Rule out an early bedtime CAUSING the early waking (sometimes moving bedtime LATER by 15–30 min solves this). Absolute darkness to prevent circadian clock triggering. Do NOT engage with the wakeful period — no lights, no play, no stimulation. Make 2am incredibly boring.

6 BABY PERSONALITY TYPES



THE SNACKER

Fueled by curiosity and tiny bites.



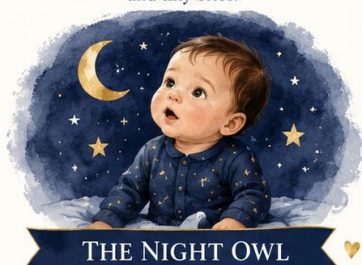
THE OVERTHINKER

Analyzing everything, even naptime.



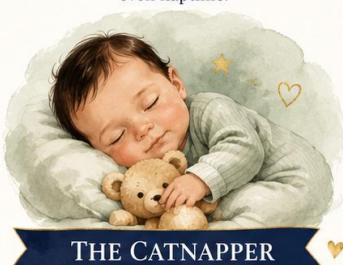
THE SENSITIVE SOUL

Feels everything deeply, loves endlessly.



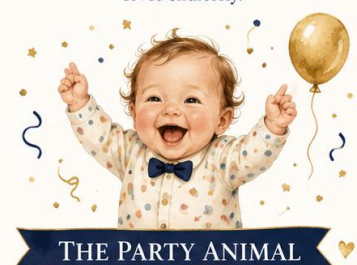
THE NIGHT OWL

Wide awake while the world sleeps.



THE CATNAPPER

Short naps, big dreams, can sleep anywhere.



THE PARTY ANIMAL

Brings joy, giggles and a little chaos.

COMBINING THE TYPES

Most babies are a BLEND of two personalities. You might have an Overthinker who's also a Sensitive Soul, or a Snacker who's also a Party Animal (baby #5 — she's in college now and still stays up too late). Use the combination to tailor your approach.

09

Common Mistakes Exhausted Parents Make (I Made Them All)

There's no shame in this chapter. These are mistakes born from desperation and love. I made every single one across my six kids.

CHAPTER NINE

Mistakes 1–4

Mistake 1: The Moving Goalpost

You start a method. Night 2 feels awful, you go in and rock to sleep. Feel guilty, try again. By Day 5 you've tried 3 approaches and your baby has no idea what's coming.

THE FIX: Pick a method and commit for at least 5–7 consecutive nights. Even the hard ones. Inconsistency is more confusing and distressing than a firm, predictable approach.

Mistake 2: Skipping the Wake Window

Desperate to get them down. Put them in the crib at the first yawn. They don't settle. You try for 45 min. They're not actually tired enough yet.

THE FIX: Track awake windows and TRUST them. A baby who isn't tired enough yet will tell you. Don't fight it — work with their biology.

Mistake 3: The Nap Trap

Long afternoon naps sound wonderful. But a baby who naps 2 hours at 4pm may not be ready for sleep at 7pm. Too much daytime sleep STEALS from the night.

THE FIX: Cap afternoon naps based on schedule needs. For most babies over 3 months, the LAST nap should END at least 2 hours before bedtime, and shouldn't be the longest nap of the day.

Mistake 4: Rescuing Too Fast

Baby stirs at 11pm. You're in there in 30 seconds because you don't want them to fully wake up. But in those 30 seconds you've INTERRUPTED a resettling process that was just beginning.

THE FIX: Wait 2–5 minutes before going in for wakings without crying. Listen carefully. Give them the chance to surprise you.

CHAPTER NINE

Mistakes 5–7

Mistake 5: Treating Weekends Differently

Weekends = flexibility. Kids up later, schedule slides. For ADULTS this is fine. For a baby in active sleep training, a 90-min schedule shift on Saturday can UNDO 5 days of progress.

THE FIX: Hold the schedule 7 days a week during active training. After 2–3 weeks of consistent independent sleep, you have more room for occasional flexibility.

Mistake 6: Waiting for Perfect Conditions

Always something. Trip next month. Holiday. Tooth that might be coming. There's NEVER a perfect time. There will always be a leap, a holiday, a cold season approaching.

THE FIX: Pick a 2-week window with no major events or travel and START. Done is infinitely better than theoretically perfect.

Mistake 7: Giving Up at the Extinction Burst

This is the sneaky one. First few nights get better. Then on Night 4 or 5, it suddenly gets WORSE. This is the EXTINCTION BURST — behavior intensifying ONE LAST TIME before it fades.

THE FIX: Know the burst is coming and PLAN for it. Write a note. Tell your partner. Put it on the fridge. 'Night 4–5 might be hard. This is normal. Do not give up.' Forewarned is forearmed.

If you've made these mistakes — including all of them — you're not failing. You're parenting. Now you have the playbook.



THE EXTINCTION BURST IN ONE SENTENCE

When something stops working (your baby's old strategy of crying-until-rescued), they try harder before they give up. Hold the line on Night 3.

10

Staying Consistent When You're Running on Empty

*Strategy only gets you so far.
Implementation is where sleep training
actually lives.*

CHAPTER TEN

Surviving the Hard Nights

Before You Start: Non-Negotiable Conversations

If you have a co-parent or anyone in nighttime care, you need to be FULLY aligned before starting. Not mostly. Fully. Sleep training done by one parent and undone by another isn't just ineffective — it's HARDER on baby than either approach done consistently.

Decide in advance: if one parent is struggling at 2am, the other holds the line. Take turns being the strong one.

The Night-by-Night Survival Plan

Here is how I survived the hard nights — practically:

- **EAR PROTECTION:** If you're not the responding parent, ear plugs or headphones playing something take the physical edge off. Sounds counterintuitive — makes you significantly more capable of staying consistent.
- **THE 'WRITE IT DOWN' RULE:** Before you go in, write down the time, how long they've been crying, whether it's escalating. Gives your rational brain something to do — and often reveals the crying is shorter than it feels.
- **THE 'GO OUTSIDE' RULE:** If you're staying back and can't take it, go outside for 60 seconds. Fresh air, brief physical separation, a moment to breathe. Helps more than you'd think.
- **THE JOURNAL:** Track each night's progress. Seeing Night 1 was 47 minutes, Night 3 was 22, Night 5 was 8 — incredibly motivating when Night 4 feels endless.

When You Slip Up

You will slip. You'll have a night where you crack and bring them into bed, or rock them fully to sleep because everyone is beyond their limit. This is human. This is parenting. This does not mean starting over from scratch.

If you slip: acknowledge it, don't catastrophize, get back on plan the very next night. ONE night doesn't undo a week. THREE nights of deviation in a row might require a restart. Be honest.



My Sleep Promise

- *I will create a calm and safe sleep environment for my baby.*
- *I will respond with patience, love, and consistency.*
- *I understand that healthy sleep takes time and practice.*
- *I will follow routines that support my baby's development.*
- *I will prioritize comfort, connection, and emotional security.*
- *I will stay flexible as my baby's needs change.*
- *I will celebrate progress, even the small wins.*
- *I will remember that better sleep is a journey, not perfection.*

Six & Thriving · Sleep, Baby, Please.

WHEN IT GETS HARD

Remember your WHY specifically. Picture in detail: a well-rested family. The hours you'll get back. The patience you'll have for your toddler. The conversations with your partner. The shower you'll take. It's coming. Keep going.

11

Toddler Sleep — When Babies Become Little Negotiators

Your baby has become a tiny, extremely opinionated, surprisingly persuasive toddler who has discovered that bedtime is negotiable. It is not.

CHAPTER ELEVEN

The Toddler Rule Book

Why Toddler Sleep Is Different

Toddler challenges are less about biology, more about development. Between 18 months and 3 years: asserting autonomy, developing theory of mind, discovering they have power. Beautiful developmentally, difficult practically.

Common disruptions: the curtain call (one more thing — water, hug, question, fear), boundary testing, nap refusal, night terrors, the 5am wake.

Limits With Warmth: The Rules

Toddlers need limits. Not because you're controlling — predictable limits create SAFETY and reduce anxiety.

ONE PASS RULE

After bedtime, your toddler gets ONE pass — one 'get out of bed free' for any reason. After it's used, they stay in bed. Hang an actual visual pass on the door for under-3s.

CONSISTENT RESPONSE

Every time they call out or come out after the pass, the SAME response: 'It's sleep time. Back to bed.' Warm, firm, boring. Same words every time.

THE GOODBYE RITUAL

Something they can count on to mark the end — special handshake, specific song. Always the same. Signals: this is actually the end, not a negotiating position.

OK-TO-WAKE CLOCK

A clock that glows green when it's okay to get up. Game-changing for early risers.



THE DOOR RULE

'If you stay in bed, the door stays open. If you come out, the door closes.' Follow through every single time. Not a punishment — a natural consequence. Most toddlers choose the open door within 3–4 nights.

12

Your First Week: A Night-by-Night Game Plan

*All right. You've got the knowledge.
Let's put it together into an actual plan
you can use starting tonight.*

CHAPTER TWELVE

Pre-Week Setup

The Day Before You Start

1. Set up the room: blackout curtains, white noise positioned and tested, temperature 68–72°F.
2. Write out your bedtime routine steps in order. Put them somewhere visible.
3. Decide bedtime target. For most babies: 7–8pm.
4. Set up a simple sleep log: date, awake times, nap times, night wakings.
5. Tell your partner / family / anyone helping. Get full alignment.
6. Do a 'nap day' — watch awake windows closely and feel out your baby's natural rhythm.

Premium Flat Roadmap Infographic

7 Night Horizontal Journey



NIGHT 1

Foundation Night

Start the routine at your target time. Calm. Consistent. Predictable. This will likely be the hardest — your baby doesn't know what's happening yet. Stay the course with your chosen method. Log everything. Tonight you're planting the seed. Expect it to take longer. Give yourself grace.

NIGHT 2

Signal Night

Your baby is starting to notice the pattern. Settling may still take a while, but you'll likely notice shorter periods before they start to quiet. Night wakings may be similar to Night 1. This is normal. Don't evaluate yet — the data isn't in. Stay consistent.

NIGHT 3

Turning Point (Maybe) — OR THE EXTINCTION BURST

Many families see a meaningful shift on Night 3. The routine has started to register. Some babies begin settling significantly faster. BUT this is also when the EXTINCTION BURST may appear — things suddenly getting WORSE before they get better. THIS IS NOT FAILURE. It is proof the old system is breaking down. Hold the line. Tomorrow will be different.

CHAPTER TWELVE

Nights 4–7

NIGHT 4

The Possible Dip

If tonight is harder than Nights 2 and 3, check the note you wrote. The extinction burst is expected. It is temporary. DO NOT change methods tonight. Stay consistent.

NIGHT 5

The Breakthrough

For many babies, this is the night you realize you haven't heard anything since 7:30pm. If this is you — congratulations. Write it down. Celebrate quietly. If not yet — still normal. Some babies take 7–10 days, especially Sensitive Souls.

NIGHT 6

Reinforcement Night

Whatever gains you've made, REINFORCE them tonight by being exactly as consistent as before. NOT the night to relax the routine or push boundaries. Pattern is forming. Protect it.

NIGHT 7

Evaluation Night

Look at your sleep log. Compare Night 7 to Night 1. What's changed? How much faster is settling? How many fewer/shorter wakings? Even if not where you hoped, there will be progress. Use the data to decide: continue same method, or adjust for week 2.

AFTER WEEK ONE

Most babies show significant improvement within 7–14 days. If at 2 weeks with minimal progress: is the method right for your baby's personality? Are you TRULY consistent? Medical factors to rule out (reflux, ear infections, allergies)? A consult with a paediatric sleep consultant can help. There's no shame in asking for help.

13

Breastfeeding & Sleep — The Honest Guide

*The intersection of breastfeeding and sleep training
is the most Googled topic in this space.
Here's the truth.*

CHAPTER THIRTEEN

Breastfeeding & Independent Sleep ARE Compatible

This is the #1 fear driving purchase hesitation in breastfeeding communities. Address directly: you do NOT have to choose between breastfeeding and sleep training. They work TOGETHER.

Nursing-to-Sleep: Association vs. Nutrition

A NUTRITIONAL FEED: Baby drinks purposefully, transfers significant volume, feeds with intent. This is real hunger.

A COMFORT FEED: Brief latch, flutter-sucking, drifts back to sleep. Baby is using the breast as a sleep prop, not eating.

The goal is to separate the nutritional feeds from the sleep-onset moment. Eliminate the comfort/habit feeds — keep the nutritional ones.

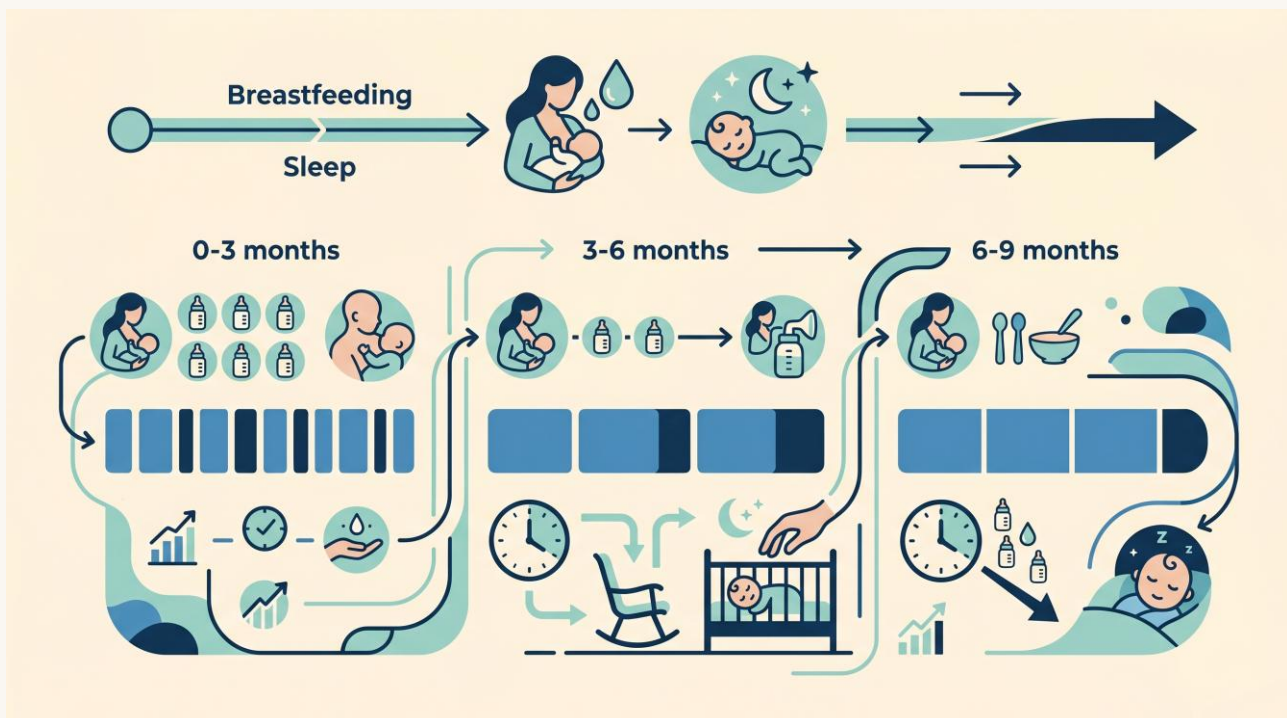
How to Gently End a Comfort Feed (The Unlatch Technique)

1. Watch for the flutter-suck (slow, gentle, no swallowing)
2. Slip your finger gently into the corner of baby's mouth
3. Break the suction softly — they will release
4. Sit baby up briefly. Rouse them just enough to be drowsy-not-asleep.
5. Place in crib awake. Calm phrase. Leave.

Dream Feeds for Breastfed Babies

A dream feed (10–11pm, without fully waking baby) can extend the first stretch of night sleep by 1–2 hours. Offer for 2–3 weeks, then gradually drop once baby is sleeping through.

How: Lights off. No talking. Gently lift baby. Latch them while they remain drowsy. Let them feed for 5–8 minutes. Place back in crib. They often don't fully wake.



CHAPTER THIRTEEN

When to Drop Night Feeds (Breastfed)

AGE	GUIDANCE	STATUS
0–4 months	Feed on demand. All nutritional.	DON'T DROP
4–6 months	Reduce to 1–2 feeds. Increase daytime calories.	REDUCE GRADUALLY
6–9 months	Most can handle 0–1 night feeds.	ALMOST THERE
9+ months	If weight gain on track, night feeds are habitual.	READY TO DROP

Your Supply Will NOT Disappear

Your body adjusts to daytime demand within 3–5 days of dropping a night feed. Pump once before bed in the first few days if concerned about engorgement.

Common Q&A

Q: Will my supply drop?

A: No. Your body adjusts within 3–5 days.

Q: Is my baby actually hungry at night?

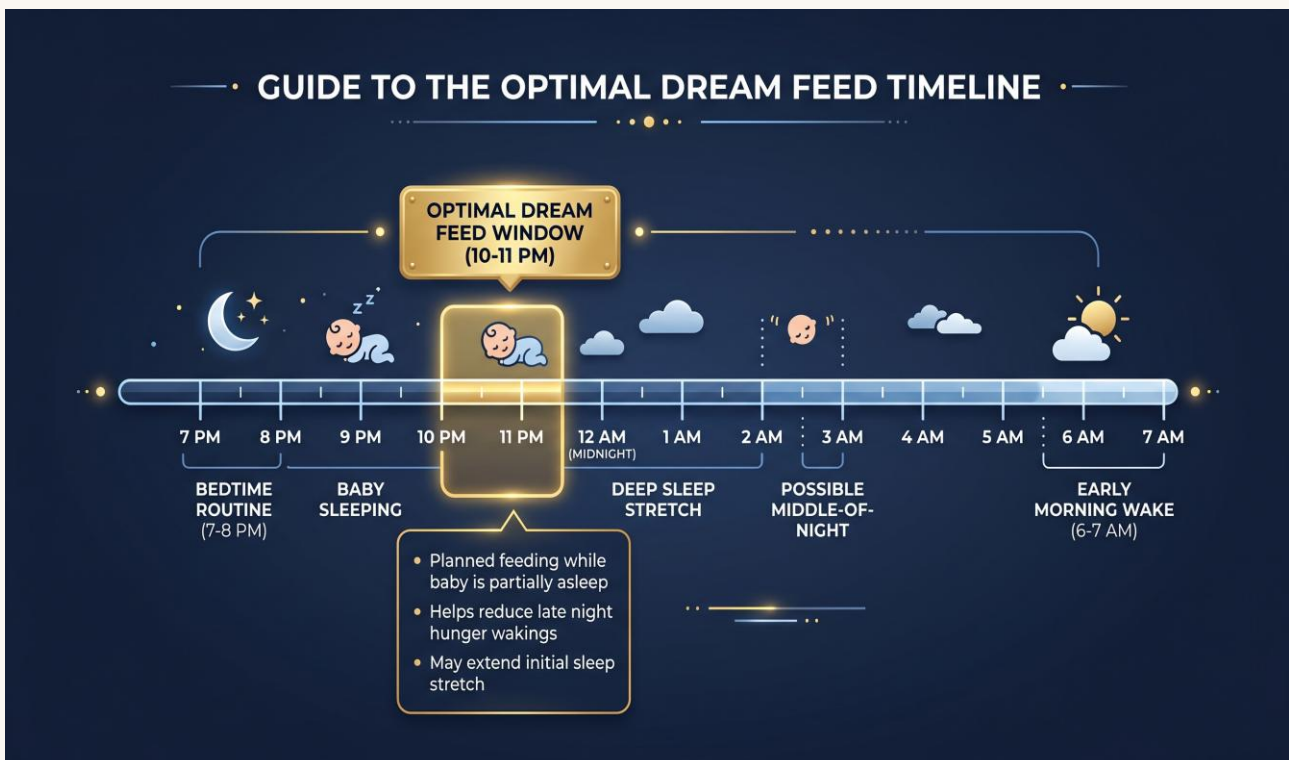
A: If they feed for less than 5 min and drift off, it's habitual.

Q: Can I still nurse to sleep for naps?

A: Ideally no — consistency matters. But bedtime is the priority. Naps can be a transition zone.

Q: My baby refuses bottles from my partner — what now?

A: Common. Have partner offer bottle when baby is HUNGRY but NOT starving (15 min before usual feed). Use a slow-flow nipple. Skin-to-skin during the bottle. Try for 7–10 days before declaring it 'not working.'



14

Twins, NICU & Daycare — Special Situations

*I have twins. I know this section is overdue.
This chapter is for the parents standard
guides ignore.*

CHAPTER FOURTEEN

Twins, Premature Babies & Daycare

Sleep Training Twins

The Golden Rule for Twins: When one wakes, wake the other. Synchronize feeds and sleep from Day 1. Train **SIMULTANEOUSLY** — same method, same night, same response.

Same room? YES. Babies habituate to each other's sounds faster than you'd expect. Separate rooms isn't necessary unless one has a medical condition.

Accept that one twin will progress faster. That's normal — they have different temperaments. Use the same method for both, and let each one's pace be their pace.

If one is sick, pause for **THAT** twin only. Continue with the other. Resume the sick twin when they recover (3–5 nights to reset).

Premature & NICU Babies

USE **CORRECTED AGE** for all sleep milestones. A baby born 6 weeks early at 4 months calendar age is developmentally 2.5 months — and their awake windows, sleep needs, and readiness for training reflect that.

Corrected age = calendar age – weeks early. Don't sleep-train until corrected age is 4–6 months at minimum, and your paediatrician confirms baby is ready.

NICU families carry fear, hypervigilance, and guilt into the sleep journey. That is **VALID**. You are not being overprotective — you are being a parent who has been through something hard. Honor that. Move at your pace.

Daycare & Nursery Schedules

ASK YOUR NURSERY: What time is the last nap? How long? This drives your bedtime.

ACCEPT: Daycare naps are often shorter — compensate with an **EARLIER** bedtime (sometimes 30 min earlier on daycare days).

WEEKENDS: Keep the **SAME** schedule. Consistency across all 7 days matters more than weekend rest.

GIVE: A one-page summary to your provider (use the Partner Briefing format in Chapter 15).



15

The First 8 Weeks — Newborn Period

*Sleep training starts at 4 months.
Until then, here is what you are building.
You are not behind.*

CHAPTER FIFTEEN

What Newborn Sleep Actually Looks Like

What Normal Newborn Sleep Looks Like

- 14–17 hours of sleep in 24 hours, in 2–4 hour chunks
- NO circadian rhythm yet — day and night are meaningless to them
- Frequent feeding is biologically necessary and expected
- 'Sleeping through' is NOT a realistic goal before 3–4 months

What You CAN Do Now (Foundation Building)

- Expose baby to natural light during the day (10–15 min near a window)
- Keep night feeds DARK, QUIET, and BORING — no eye contact, no talking
- Start a simple bedtime routine from week 3–4 (bath → feed → swaddle → crib)
- Practice ONE crib nap per day (even if it's short)
- Distinguish between active sleep sounds and actual waking — many newborn 'wake' sounds are just sleep cycling

The SNOO & Swaddle Question

Swaddles are appropriate UNTIL baby shows signs of rolling (typically 3–4 months). Stop swaddling immediately when rolling starts — it becomes a SIDS risk.

The SNOO is a tool, not a crutch — but plan your transition off it BEFORE 5 months to avoid creating a motion dependency that's harder to undo later.

Common Newborn Sleep Mistakes

- Trying to sleep-train before 4 months (too early)
- Keeping nights bright/loud (delays circadian development)
- Not feeding frequently enough (leads to overtiredness AND undernutrition)
- Comparing your newborn to other babies (every newborn is different)
- Dropping naps early (newborns need 4–5 naps; cutting them = overtiredness)

HOLD THIS CLOSE

You are not behind. You are not failing. You are building the foundation that makes everything else possible. Some of the most important sleep work in the first 8 weeks looks like 'just feeding and surviving.' That IS the work.

PARTNER BRIEFING

The One-Pager for the Person Who Hasn't Read the Book

Print this. Give it to your partner. Consistency requires two people.

THE PLAN (in 60 seconds):

1. Bedtime routine: same steps, same order, every night
2. Baby goes into crib drowsy but awake
3. When baby cries: we wait [X] minutes before responding
4. Our response method is: _____
5. We do the SAME thing every time — no exceptions for 7 nights

WHY THIS MATTERS:

If one of us holds firm and the other caves at 2am, we teach baby that escalating works. This makes the crying WORSE, not better. Consistency between both of us is the single biggest predictor of success.

NIGHT 3 WARNING:

Night 3 will be the hardest. This is the EXTINCTION BURST — it means the method is WORKING, not failing. DO NOT change the plan on Night 3. Night 4 is almost always dramatically better.

IF YOU'RE DOING THIS ALONE:

Single parents carrying this weight alone: you are doing something extraordinary. Write your plan down. Read it at 2am when your resolve wavers. You are both parents in one — and that is enough.



My Sleep Promise

- ✦ *For the next seven days, I will keep bedtime simple and in the same order.*
- ✦ *I will watch awake windows and act on them before overtiredness sets in.*
- ✦ *I will decide my response plan before the night begins.*
- ✦ *I will pause before I go in — and give my baby the chance to resettle.*
- ✦ *I will not judge the whole plan on one difficult night.*
- ✦ *I will stay consistent — especially when it is hardest.*
- ✦ *I will remember I am teaching a skill they will use for life.*
- ✦ *I will take care of myself — my baby needs a rested parent, not a perfect one.*

With love and solidarity,

Six & Thriving

Sleep is coming. For both of you.

CONCLUSION

You've Got This — And Your Baby Will Sleep

We started this book in the dark. Maybe literally. Maybe you were in that rocking chair, or sitting in the hallway listening, or lying in bed staring at the ceiling waiting for the next waking. And we end here, with a plan in your hands and a little more understanding in your heart.

What You've Learned

- Your baby's sleep challenges are biology, development, and habit — NOT your fault
- Sleep science gives you real tools: awake windows, circadian rhythms, sleep pressure
- Reading cues before overtiredness changes everything
- A consistent routine is one of the most powerful sleep tools you have
- The environment is doing work all night — make sure it's working FOR you
- The best method is the one you can commit to consistently
- Night 3 is the extinction burst — and it means the method is working
- Knowing your baby's sleep personality helps you tailor your approach
- Common mistakes are avoidable once you know what they are
- Consistency on hard nights is the bridge between where you are and where you want to be

The Most Important Thing I Can Tell You

Across six children, across the exhausted nights and the mornings that felt impossible — every single one of them learned to sleep. Every single one. The stubborn ones. The sensitive ones. The ones who seemed physiologically opposed to sleep. They all got there.

And so will yours.

HOLD THIS CLOSE

A baby who sleeps well is a baby who thrives. A parent who sleeps is a parent who's fully present. Sleep is not a luxury — it is a foundation. Go build that foundation. I'm cheering for you.

BONUS

Quick-Reference Checklists & Tools

*Tear-out ready. Print them. Stick them to the fridge.
Use them on the hard nights when reading feels like too much.*

BONUS ONE

Bedtime Routine Checklist

Same order. Every night. Consistency IS the method.

- ☐ Transition signal — lights dimmed, specific phrase spoken
- ☐ Stimulating activities stopped — screens off, active play ended
- ☐ Bath or warm wipe-down completed
- ☐ Lotion massage — slow, calming strokes
- ☐ Pyjamas and sleep sack on
- ☐ Feed in dim, quiet room — baby must NOT fall fully asleep
- ☐ 1–3 books or songs in nursing chair / glider
- ☐ Final goodnight phrase (same words every night)
- ☐ White noise on (60–70dB, across the room)
- ☐ Room fully dark — all LEDs covered, true blackout
- ☐ Baby placed in crib drowsy but awake
- ☐ Parent exits calmly and consistently

Before You Go In



(1) Put in earplugs. Step back.



(2) Write down the time and whether it's escalating.



(3) Go outside for 60 seconds. Breathe.



(4) Check your log. Night 1 was 47 min. You're improving.

Your 2am brain will thank your 2pm brain.

BONUS TWO

7-Day Sleep Tracker

Fill in each morning. Patterns emerge by Day 3–4.

Day	Last Nap	Bedtime	Settle	Wakes	Up	Notes
Day 1						
Day 2						
Day 3						
Day 4						
Day 5						
Day 6						
Day 7						

Safe Sleep Checklist (AAP-Aligned)

SURFACE

- ☐ Firm flat mattress
- ☐ Fitted sheet only
- ☐ No toys, bumpers, positioners
- ☐ Sleep sack used
- ☐ Back to sleep — every time
- ☐ Crib/bassinet only

ROOM

- ☐ Room 68–72°F
- ☐ Chest warm not sweaty
- ☐ True blackout
- ☐ White noise running
- ☐ Monitor LEDs covered
- ☐ No nightlight, or red only

BONUS THREE

Awake Window Quick Reference

Pull this out when you need it.

AGE	AWAKE WINDOW	MAX NAPS	BEDTIME TARGET
0–6 weeks	45–60 min	4–5/day	Variable
6–12 weeks	60–90 min	4/day	8–9 pm
3–4 months	90 min	3–4/day	7:30–8:30 pm
4–6 months	2 hrs	3/day	7–8 pm
6–8 months	2.5 hrs	2–3/day	7–8 pm
8–12 months	3–3.5 hrs	2/day	6:30–7:30 pm
12–18 months	4–5 hrs	1–2/day	7–7:30 pm
18–24 months	5–6 hrs	1/day	7–7:30 pm

ONE LAST REMINDER

Count FORWARD from wake time, not BACKWARD from a target. If your baby's last nap ended at 5pm and the awake window is 3 hours, bedtime is 8pm — even if 'we always do 7'.

BONUS FOUR

Method Comparison Card

Which Method Fits Your Family? Choose. Commit. Don't second-guess.

METHOD	CRY	SPEED	BEST FOR
Extinction (CIO)	High	3–5 days	High-need families; persistent babies; need fast results
Ferber	Moderate	5–7 days	Most temperaments; parents wanting to do something
Chair Method	Low–Mod	2–4 weeks	Sensitive babies; parents not ready to step back
Fading	Low	3–4+ wks	Gentlest; adaptable babies; no-cry priority
Pick Up/Put Down	Low	Variable	3–5 month olds; high-support preference

The Decision Shortcut

If you need fast results AND can stay consistent → Extinction

If you need 'something to do' during crying → Ferber

If your baby is very sensitive → Chair Method

If you want zero crying → Fading

If baby is 3–5 months → Pick Up / Put Down

Pick ONE method. Commit. Stick to it for 7 nights minimum before evaluating. The method matters less than the consistency.



ABOUT THE AUTHOR

Six & Thriving

Six & Thriving is the pen name of a mother of six — including twins — who has spent a decade in the trenches of infant and toddler sleep.

This book was written in the quiet hours, fueled by cold coffee, hard-won experience, and the conviction that sleep is possible for every family.

Six & Thriving creates practical, evidence-informed content for exhausted parents who need real answers, not gentle theory.

"Every single one of my babies learned to sleep. Every single one."

— Six & Thriving



Thank you for trusting Six & Thriving with your baby's sleep journey.

Sleep is coming. For both of you.

Disclaimer: Educational purposes only. Not medical advice. Consult your paediatrician.

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